

Scenario: Management of children and young people with type 2 diabetes

Last revised in March 2025

From age 12 months to 17 years.

What initial information and advice should I offer a child or young person with type 2 diabetes?

Ensure that the child or young person has an individualized care plan, provided by a multidisciplinary paediatric diabetes team.

- Ensure that the child and their family/carers know how to contact the specialist diabetes team and are involved in making decisions about their care.
 - The child or young person may be offered home-based or inpatient management initially, depending on clinical need and family circumstances and wishes.
- Ensure that the child and their family/carers are offered a continuing programme of education, which is tailored to their individual needs, including:
 - The need for HbA1c monitoring and [treatment targets](#) ([/topics/diabetes-type-2/management/management-children-young-people/#treatment-targets](#)).
 - The effects of [lifestyle](#) ([/topics/diabetes-type-2/management/management-children-young-people/#lifestyle-advice](#)), and [intercurrent illness](#) ([/topics/diabetes-type-2/management/management-children-young-people/#managing-intercurrent-illness](#)) on blood glucose control.
 - The aims of treatment with [antidiabetic drugs](#) ([/topics/diabetes-type-2/management/management-children-young-people/#antidiabetic-drugs](#)) and possible adverse effects.
 - The need for regular [monitoring for complications](#) ([/topics/diabetes-type-2/management/management-children-young-people/#screening-for-managing-complications](#)) of type 2 diabetes, and how to reduce the risk. Note: this will usually be arranged by the paediatric diabetes team at diagnosis and regularly thereafter. If there is any uncertainty, seek specialist advice.
- Provide advice on sources of information and support.
 - Diabetes UK (website available at www.diabetes.org.uk (<https://www.diabetes.org.uk/>)) — a national charity that provides information, support, and advocacy for people with diabetes and their families. It has a confidential helpline (0345 123 2399) and hosts an online community for peer support, as well as a wide range of education and information [patient resources](#) (<https://www.diabetes.org.uk/professionals/information-support-for-your-patients/education-information>).
 - For detailed information on managing other lifestyle and cultural issues, such as driving, fasting, holiday and travel, insurance, and shift work, see the CKS topic on [Insulin therapy in type 2 diabetes](#) ([/topics/insulin-therapy-in-type-2-diabetes/](#)).
- Assess for anxiety, depression, emotional and behavioural difficulties, and eating disorders, and offer emotional and psychosocial support as needed. See the CKS topics on [Depression in children](#) ([/topics/depression-in-children/](#)), [Conduct disorders in](#)

[children and young people \(/topics/conduct-disorders-in-children-young-people/\)](/topics/conduct-disorders-in-children-young-people/), and [Eating disorders \(/topics/eating-disorders/\)](/topics/eating-disorders/) for more information.

- The Diabetes UK patient resource [Diabetes and your emotions \(https://www.diabetes.org.uk/guide-to-diabetes/emotions\)](https://www.diabetes.org.uk/guide-to-diabetes/emotions) may be helpful.
- Advise about sexual health, contraception, and the importance of pre-pregnancy counselling, if appropriate. See the CKS topics on [Contraception - assessment \(/topics/contraception-assessment/\)](/topics/contraception-assessment/) and [Pre-conception - advice and management \(/topics/pre-conception-advice-management/\)](/topics/pre-conception-advice-management/) for more information.
 - The Diabetes UK patient resource [Planning for a pregnancy when you have diabetes \(https://www.diabetes.org.uk/guide-to-diabetes/life-with-diabetes/pregnancy\)](https://www.diabetes.org.uk/guide-to-diabetes/life-with-diabetes/pregnancy) may be helpful.
- Offer immunization against influenza (if over the age of 6 months) and pneumococcal infection (if needing oral antidiabetic drugs or insulin). See the CKS topics on [Immunizations - seasonal influenza \(/topics/immunizations-seasonal-influenza/\)](/topics/immunizations-seasonal-influenza/) and [Immunizations - pneumococcal \(/topics/immunizations-pneumococcal/\)](/topics/immunizations-pneumococcal/) for more information.
- Measure height and weight, calculate body mass index (BMI), and plot on an appropriate growth chart.
 - Check for normal growth and/or significant changes in weight, as these may reflect changes in blood glucose control.
- Advise the person to always wear or carry some form of diabetes identification, such as a:
 - MedicAlert® bracelet, necklace, or watch (available from [www.medicalert.org.uk \(http://www.medicalert.org.uk/\)](http://www.medicalert.org.uk)).
 - Diabetes identity card or wristband (available from [www.diabetes.org.uk \(https://www.diabetes.org.uk/\)](https://www.diabetes.org.uk)).

Basis for recommendation

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What are the treatment targets for children and young 2 diabetes?

- The recommended HbA1c target level for children and young people with type 2 diabetes is 48 mmol/mol (6.5%) or lower, to minimize the risk of long-term complications (</topics/diabetes-type-2/background-information/complications/>).
 - Agree an individualized lowest achievable HbA1c target with each child or young person and family/carers, taking into account factors such as daily activities, individual life goals, complications, co-morbidities, and the risk of [hypoglycaemia \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/).
 - Support them to safely achieve and maintain their individually agreed HbA1c target level.
 - If the HbA1c level is above the ideal target, explain that any reduction in HbA1c reduces the risk of long-term complications.
 - Ensure that children and young people with type 2 diabetes are offered measurement of their HbA1c level every 3 months.

Basis for recommendation

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Which antidiabetic drugs are available for children and with type 2 diabetes?

- Antidiabetic drug treatment for children and young people with type 2 diabetes should be initiated by the multidisciplinary paediatric diabetes team.
 - Standard-release metformin should be offered from diagnosis to children and young people with type 2 diabetes, and insulin therapy may be needed in some cases. See the CKS topic on [Insulin in type 2 diabetes \(/topics/insulin-therapy-in-type-2-diabetes/\)](#) for more information.

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What lifestyle advice should I give a child or young person with type 2 diabetes?

Diet

Ensure the child or young person has been offered appropriate, individualized, and ongoing nutritional advice from a healthcare professional with specific expertise and competencies in nutrition.

- **Emphasize the importance of a healthy, balanced diet:**
 - Encourage the child or young person to eat at least 5 portions of fruit and vegetables each day.
 - Encourage high fibre, low-glycaemic-index sources of carbohydrate (such as fruit, vegetables, wholegrains, and pulses), low-fat dairy products, and oily fish.
 - Portion sizes should be appropriate for age, stage of growth, and energy requirements.
 - Advise controlling the intake of foods containing saturated and trans-fatty acids, high-sugar drinks, and foods high in salt content.
 - Emphasize the importance of meal-time routines and to minimize snacking.
 - The Diabetes UK patient resources [Weight loss and diabetes \(/https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/eating-with-diabetes/whats-your-healthy-weight/lose-weight\)](#) and [What is a healthy, balanced diet for diabetes? \(/https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/eating-with-diabetes/what-is-a-healthy-balanced-diet\)](#) may be helpful.
- **Offer dietary advice with other aspects of lifestyle modification, such as [exercise \(/topics/diabetes-type-2/management/management-children-young-people/#exercise-physical-activity\)](#) and weight loss, as appropriate.**

Exercise and physical activity

- **Provide information on regular exercise and physical activity.**
 - The Diabetes UK patient resource [Diabetes and exercise \(/https://www.diabetes.org.uk/guide-to-diabetes/managing-your-diabetes/exercise\)](#) provides helpful information on the type and duration of recommended exercise.
 - Aim to minimize time spent being sedentary.
 - Advise that regular exercise may lower blood glucose levels, improve cardiovascular risk, and reduce excess weight (when combined with a [healthy diet](#)

[\(/topics/diabetes-type-2/management/management-children-young-people/#diet\)](/topics/diabetes-type-2/management/management-children-young-people/#diet)).

- **Provide information on the following (if appropriate):**
 - The potential interaction of exercise and alcohol. Advise seeking advice from their specialist diabetes team if they wish to participate in a sport that may have particular risks for people with diabetes.
 - The effect of exercise on blood glucose levels when insulin levels are adequate (risk of hypoglycaemia [\(/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/)).
 - The effect of exercise on blood glucose levels when the person is hyperglycaemic (risk of worsening blood glucose control and ketones in the blood).
 - The appropriate adjustments of insulin dosage and/or nutritional intake for exercise and post-exercise periods. See the CKS topic on [Insulin in type 2 diabetes](/topics/insulin-therapy-in-type-2-diabetes/) [\(/topics/insulin-therapy-in-type-2-diabetes/\)](/topics/insulin-therapy-in-type-2-diabetes/) for more information.

Alcohol intake

- **Provide information on recommended alcohol limits and the potential impact of alcohol on diabetes and its treatment.**
 - Provide information that the legal drinking age in the UK is 18 years and advise on the recommended alcohol limits.
 - The national charity Drinkaware (website available at www.drinkaware.co.uk [\(https://www.drinkaware.co.uk/\)](https://www.drinkaware.co.uk/)) has useful information on [The law on alcohol and under 18s](https://www.drinkaware.co.uk/facts/alcohol-and-the-law/the-law-on-alcohol-and-under-18s) [\(https://www.drinkaware.co.uk/facts/alcohol-and-the-law/the-law-on-alcohol-and-under-18s\)](https://www.drinkaware.co.uk/facts/alcohol-and-the-law/the-law-on-alcohol-and-under-18s).
 - Advise the person to ideally eat a snack that contains carbohydrate before and after drinking alcohol.
 - Educate that alcohol may exacerbate or prolong the hypoglycaemic effect of insulin therapy.
 - The signs of [hypoglycaemia](/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/) [\(/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/) may become less obvious, and delayed hypoglycaemia may occur up to several hours after alcohol consumption. See the CKS topic on [Insulin therapy in type 2 diabetes](/topics/insulin-therapy-in-type-2-diabetes/) [\(/topics/insulin-therapy-in-type-2-diabetes/\)](/topics/insulin-therapy-in-type-2-diabetes/) for more information on the possible presentation and effects of hypoglycaemia.
 - Advise the person to always wear or carry some form of diabetes identification, as reduced awareness of hypoglycaemia may be confused with alcohol intoxication, such as a:
 - MedicAlert® bracelet, necklace, or watch (available from www.medicalert.org.uk [\(http://www.medicalert.org.uk/\)](http://www.medicalert.org.uk/)).
 - Diabetes identity card or wristband (available from www.diabetes.org.uk [\(https://www.diabetes.org.uk/\)](https://www.diabetes.org.uk/)).
 - The Diabetes UK patient resource [Alcohol and diabetes](https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/what-to-drink-with-diabetes/alcohol-and-diabetes) [\(https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/what-to-drink-with-diabetes/alcohol-and-diabetes\)](https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/what-to-drink-with-diabetes/alcohol-and-diabetes) may be helpful.

Smoking and drug misuse

- **Provide information on the impact of smoking on diabetes and its complications, such as cardiovascular disease.**
 - Advise on smoking cessation, if appropriate. See the CKS topic on [Smoking cessation](/topics/smoking-cessation/) [\(/topics/smoking-cessation/\)](/topics/smoking-cessation/) for more information.
 - Advise young people who are non-smokers never to start smoking.

- Provide information on the risks of drug misuse and the possible effects on blood glucose control.
 - The Diabetes UK patient resource [Recreational drugs and diabetes](https://www.diabetes.org.uk/guide-to-diabetes/young-adults/drugs) (<https://www.diabetes.org.uk/guide-to-diabetes/young-adults/drugs>) may be helpful.

Basis for recommendation

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How should I manage a child or young person with a suspected hyperglycaemic emergency?

Be aware that [diabetic ketoacidosis \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/) (DKA) is more common in people with type 1 diabetes, but can occur in people with type 2 diabetes, and may present with a normal or near normal blood glucose level. [Hyperosmolar hyperglycaemic state \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/) (HHS) typically presents without significant blood or urinary ketones on testing.

- Arrange emergency admission to a hospital with acute paediatric facilities, for confirmation of the diagnosis and emergency treatment.
- After the child or young person has recovered from an episode of DKA or HHS:
 - Discuss the [precipitating factors \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/#precipitating-factors\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/#precipitating-factors) that may have led to the episode. Consider the possibility of non-adherence to treatment, particularly if there is a history of recurrent DKA.
 - Provide information and advice about how to manage intercurrent illness and 'sick-day rules', to reduce the risk of future episodes. See the section on [Managing intercurrent illness \(/topics/diabetes-type-2/management/management-children-young-people/#managing-intercurrent-illness\)](/topics/diabetes-type-2/management/management-children-young-people/#managing-intercurrent-illness) for more information.

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How should I manage intercurrent illness in a child or young person with type 2 diabetes?

If a child or young person with type 2 diabetes is unwell, consider the need to arrange hospital admission or seek specialist advice, depending on clinical judgement, taking into account the person's age, co-morbidities, risk of complications, and the presence of hyperglycaemia and/or ketosis.

- Arrange immediate hospital admission if:
 - There is an immediate risk of [diabetic ketoacidosis \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/) (DKA), such as ketonaemia (1.5–2.9 mmol/L) with or without hyperglycaemia, and the child cannot eat or drink.
 - There is an immediate risk of [hyperosmolar hyperglycaemic state \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/) (HHS).
 - There is suspected acute kidney injury (AKI) that cannot be managed in primary care. See the CKS topic on [Acute kidney injury \(/topics/acute-kidney-injury/\)](/topics/acute-kidney-injury/) for more

information.

- A person treated with insulin does not show signs of clinical improvement with insulin treatment. See the CKS topic on [Insulin therapy in type 2 diabetes \(/topics/insulin-therapy-in-type-2-diabetes/\)](/topics/insulin-therapy-in-type-2-diabetes/).
- **Consider arranging hospital admission or seeking urgent specialist advice if:**
 - The underlying condition is unclear.
 - The child is dehydrated or at risk of dehydration.
 - Vomiting persists beyond 2 hours (particularly in young children).
 - The child is younger than 3 years of age or has a co-existing medical condition.
 - The person and their family/carers are unable to keep the blood glucose level above 3.5 mmol/L.
 - The person and their family/carers are exhausted, for example due to repeated night-time waking.
- **If admission is not needed and the child or young person can be managed in primary care:**
 - Ensure that the person or family/carers has written contact details of their specialist diabetes team.
 - Assess the child and manage any intercurrent illness(es), as appropriate.
 - Provide the person or family/carers with clear, individualized advice on 'sick-day rules' (</topics/diabetes-type-2/management/management-children-young-people/#sick-day-rules>), to manage diabetes during episodes of intercurrent illness or hyperglycaemia, and reinforce the advice regularly.
 - Advise that intercurrent illness may affect blood glucose control, and there is a risk for worsening hyperglycaemia even if dietary intake is reduced.
 - Ensure the child has sick day foods and drinks supplies readily accessible at home, including:
 - Easily digestible foods and sugary drinks (to provide energy and to prevent further ketosis).
 - Over-the-counter oral rehydration therapy (ORT) sachets (to prevent dehydration).
 - Glucose tablets or oral gel (to prevent hypoglycaemia).
 - Equipment for monitoring blood glucose and ketones (if appropriate).
 - Additional supplies of insulin (if appropriate).
 - A glucagon kit (if appropriate).
 - Note: see the CKS topic on [Insulin therapy in type 2 diabetes \(/topics/insulin-therapy-in-type-2-diabetes/\)](/topics/insulin-therapy-in-type-2-diabetes/) for information on blood glucose and ketone monitoring meters, glucose oral gels, and other accessories.

'Sick-day rules'

- **Reinforce 'sick-day rules' advice on managing episodes of intercurrent illness, which should have been provided by the child's specialist diabetes team.**
 - If on insulin therapy, do not stop treatment. See the CKS topic on [Insulin therapy in type 2 diabetes \(/topics/insulin-therapy-in-type-2-diabetes/\)](/topics/insulin-therapy-in-type-2-diabetes/) for more information.
 - Advise the dose of insulin may need to be altered during periods of illness. Seek advice from the specialist diabetes team if there is uncertainty on how to adjust insulin doses.
 - Consider the need for blood [ketone monitoring \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/), depending on advice from the specialist diabetes team.
 - This should be checked regularly, for example at least every 3–4 hours including through the night and advice given to record the results.

- If the blood ketone level is greater than 3 mmol/L, the person or family/carers should seek immediate medical advice. See the section on [When to suspect hyperglycaemic emergencies \(DKA and HHS\) \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hyperglycaemic-emergencies-dka-hhs/\)](#) for more information.
- Advise to maintain their normal meal pattern (including fluids and carbohydrate intake) where possible if appetite is reduced.
 - If unable to eat or vomiting, advise to replace normal meals with carbohydrate-containing drinks (such as milk, milkshakes, fruit juices, and sugary drinks).
 - If blood glucose levels are high, maintain fluid intake with sugar-free fluids.
 - If blood glucose levels are low, encourage regular intake of sugary fluids.
- Advise to seek urgent medical advice if the child:
 - Is unable to eat or drink, is dehydrated or at risk of dehydration.
 - Has persistent vomiting.
 - Has [hypoglycaemia \(/topics/diabetes-type-2/diagnosis/when-to-suspect-hypoglycaemia/\)](#) that cannot be managed in primary care.
- The Diabetes UK information [Diabetes when you're unwell \(/https://www.diabetes.org.uk/guide-to-diabetes/life-with-diabetes/illness\)](#) may be helpful.

[[Down, 2020 \(/topics/diabetes-type-2/references/\)](#)]

Basis for recommendation

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How should I screen for and manage complications in c young people with type 2 diabetes?

Retinopathy

Ensure that from the age of 12 years, all children with type 2 diabetes attend for diabetic eye screening, as part of the NHS retinopathy screening programme.

- Children should attend for an eye examination by an optometrist every 2 years until the age of 12 years.
 - Children should be referred to the local retinopathy screening programme prior to their twelfth birthday, to ensure that they are seen from 12 years of age.
 - Consider referring children who are younger than 12 years to an ophthalmologist for retinal examination, if blood glucose control is suboptimal.
- Explain that retinal screening aims to reduce the risk of sight loss by the early detection and treatment, if needed, of sight-threatening retinopathy. Improving blood glucose control will reduce the risk of any background retinopathy detected progressing to significant diabetic retinopathy.
 - The Diabetes UK patient resource [Diabetic eye screening \(/https://www.diabetes.org.uk/guide-to-diabetes/managing-your-diabetes/diabetic-eye-screening\)](#) may be helpful.

Foot problems

- **For children with type 2 diabetes who are younger than 12 years of age, explain the importance of regular foot care and give basic foot care advice:**

- Explain the need to check daily the entire surface of both feet, including areas between the toes. The Diabetes UK patient resource [Diabetes and foot problems](https://www.diabetes.org.uk/guide-to-diabetes/complications/feet) (<https://www.diabetes.org.uk/guide-to-diabetes/complications/feet>) may be helpful.
- Foot emergencies and who to contact.
- Footwear advice, including to avoid shoes that are too tight, have rough edges, or uneven seams; to avoid tight-fitting socks and change socks daily.
- Nail-cutting advice.
- How to recognize foot ulcers and pre-ulcerative signs.
- **For young people with type 2 diabetes aged between 12–17 years**, ensure that the specialist diabetes team arrange a foot check at least once a year, or sooner if any foot problems arise.
 - If a diabetic foot problem is found or suspected, referral should be arranged to an appropriate specialist.

Diabetic kidney disease

Ensure all children and young people with type 2 diabetes have annual screening for diabetic kidney disease.

- **To assess for diabetic kidney disease in children:**
 - Arrange an early morning first-void urine sample if possible, for assessment of microalbuminuria by estimation of the albumin:creatinine ratio (ACR).
 - Note: a random urine sample may be used, but this is associated with an increased risk of false positive results.
 - If the initial urine ACR is 3–30 mg/mmol, confirm the result by repeating the test on two further occasions, using early morning first-void urine samples, before arranging further investigation or referral.
 - If the urine ACR is 30 mg/mmol or more, arrange further investigation or referral.
 - Liaise with the child's specialist diabetic team or paediatric nephrologist if there is any uncertainty about managing results.
- **Advise that if microalbuminuria (urine ACR is 3–30 mg/mmol) is detected**, improving blood glucose control should reduce the risk of progression to significant diabetic kidney disease.
 - The Diabetes UK patient resource [Diabetic nephropathy \(kidney disease\)](https://www.diabetes.org.uk/guide-to-diabetes/complications/kidneys_nephropathy) (https://www.diabetes.org.uk/guide-to-diabetes/complications/kidneys_nephropathy) may be helpful.

Hypertension

Ensure that all children and young people with type 2 diabetes are screened for hypertension at diagnosis and then annually.

- **When measuring blood pressure in a child or young person:**
 - Use a cuff of the correct size for the person.
 - If repeated resting blood pressure measurements are greater than the 95th percentile for age, sex, and height, confirm a diagnosis of hypertension using 24-hour ambulatory blood pressure monitoring (ABPM) before further investigation and specialist treatment.
 - Seek specialist advice if antihypertensive drug treatment is being considered.
- **Advise that if hypertension is detected**, early specialist treatment should reduce the risk of complications.
 - The Diabetes UK patient resource [Diabetes and blood pressure](https://www.diabetes.org.uk/guide-to-diabetes/managing-your-diabetes/blood-pressure) ([https://www.diabetes.org.uk/guide-to-diabetes/managing-your-diabetes/blood-](https://www.diabetes.org.uk/guide-to-diabetes/managing-your-diabetes/blood-pressure)

[pressure](#)) may be helpful.

Dyslipidaemia

Ensure that all children and young people with type 2 diabetes are screened for dyslipidaemia at diagnosis once blood glucose control is achieved, and then annually.

- **When screening for dyslipidaemia in a child or young person:**
 - Ensure that total cholesterol, high-density lipoprotein (HDL) cholesterol, non-HDL cholesterol, and triglyceride concentrations are measured.
 - Confirm dyslipidaemia using a repeat sample (fasting or non-fasting) before deciding on further management.
 - Seek specialist advice if lipid-lowering drug treatment is being considered.
- **Advise that if dyslipidaemia is detected,** early specialist treatment should reduce the risk of complications.
 - The Diabetes UK patient resource [Cholesterol and diabetes](https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/eating-with-diabetes/managing-other-medical-conditions/cholesterol-and-diabetes) (<https://www.diabetes.org.uk/guide-to-diabetes/enjoy-food/eating-with-diabetes/managing-other-medical-conditions/cholesterol-and-diabetes>) may be helpful.

Basis for recommendation

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How should I support the transition from paediatric to services?

- **Explain to young people with type 2 diabetes who are preparing for transition to adult services that some aspects of diabetes care will change at transition,** such as clinic time and location, and their diabetes specialist team.
 - Be aware that a deterioration in blood glucose control may be seen in young people due to a variety of factors including:
 - Endocrine changes leading to increased insulin resistance.
 - Erratic meal and exercise patterns. See the section on [Lifestyle advice](#) (</topics/diabetes-type-2/management/management-children-young-people/#lifestyle-advice>) for more information.
 - Poor adherence to treatment regimens.
 - Eating disorders. See the CKS topic on [Eating disorders](#) (</topics/eating-disorders/>) for more information.
 - Hazardous and risk-taking behaviours. See the section on [Lifestyle advice](#) (</topics/diabetes-type-2/management/management-children-young-people/#lifestyle-advice>) for more information.
 - The Diabetes UK patient resources [Talking to your teenager about diabetes](#) (<https://www.diabetes.org.uk/guide-to-diabetes/your-child-and-diabetes/top-teen-tips>) and [Transitioning into adult diabetes clinics](#) (<https://www.diabetes.org.uk/guide-to-diabetes/young-adults/moving-clinics>) may be helpful.

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